

Urethropubic Fistula (Pubic Bone Fistula)

Patient Information · When urine leaks into the pubic bone, causing pain and infection

WARWIKI

A urethropubic fistula is a tunnel that lets urine leak from the bladder or urethra into the **pubic bone** at the front of the pelvis. It is also called a pubosymphyseal or urosymphyseal fistula. The bone often becomes infected. This causes bad pain in the groin and trouble walking. It is rare and serious, but it can be treated.

About This Condition

Urine that leaks onto the pubic bone causes a bone infection (**osteomyelitis**) and damages the joint. Common signs:

- **Bad pain in the pubic area, groin or pelvis**, often worse when you walk
- Trouble walking, or a waddling walk
- Urine infections that keep coming back

At first, the pain is often mistaken for a joint or muscle problem. This can delay the right diagnosis for months.

What Causes It

Most men who get it had **radiation for prostate cancer**, often years earlier. Radiated tissue heals poorly. Many then had a scope procedure to open a narrowing at the bladder outlet. Urine can then leak through the weak tissue into the bone.

Results in Numbers

about
19 in 20

men who had radiation do not heal with a catheter and antibiotics alone

about
6 in 7

operations after radiation remove the bladder; some men keep it

4 vs 23 in 100

had the fistula come back with repair within about 3 months vs later (one study)

Numbers are averages from published studies. Your own risk may differ; ask your team.

Learn the Terms

Pubic symphysis

The joint where the two pubic bones meet at the front of the pelvis.

Fistula

An abnormal tunnel, here between the urine tract and the bone.

Osteomyelitis

Infection of the bone.

MRI

The best scan to show the bone and the fistula.

Debridement

Surgery to remove infected bone and tissue.

Urinary diversion

A new path for urine, away from the damaged area, often with a stoma.

Flap

Healthy tissue with its own blood supply, moved in to help the area heal.

Why is this treated with such big surgery? The problem is infected bone and tissue that heal poorly, often because of radiation. A simple patch rarely works. Lasting relief usually means removing the infected bone and sending urine away from the area. For many men, surgery finally ends the pain.

How It Is Diagnosed

- An **MRI** of the pelvis, the best test for the bone
- A look inside the bladder and urethra (**cystoscopy**)
- Tests to find the germ, sometimes from a sample of the bone

How It Is Treated

- 1 Antibiotics** chosen for the germ in the bone, often for a long time.
- 2 Surgery to remove infected bone** and the fistula.
- 3 A new path for urine:** many men need the bladder removed and a urinary diversion. Some can keep the bladder.
- 4 A flap** of healthy tissue, such as belly fat or muscle, fills the space.

Several teams often work together, such as urology, infection doctors and bone surgeons. Recovery can be long, and some men need more surgery later.

Call the office that is treating you if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Groin or pubic pain that is getting worse
- New trouble walking

Go to an emergency department if:

- Fever with shaking chills and feeling very ill
- You cannot urinate, or the catheter stops draining
- You cannot reach the office and are worried

Three Things to Remember

- 1.** New pubic or groin pain after prostate radiation can be this condition. Tell your team about the radiation.
- 2.** An MRI makes the diagnosis.
- 3.** Treatment is big surgery, but it can end the pain. Earlier repair seems to work better.